

THE KAJIDORI COLLECTIVE PRESENTS

The 2026 CQC Compliance Checklist

For Care Home Managers & Registered Managers

A practical, inspector-tested framework covering all five CQC key questions. Use this before your next inspection to identify gaps, evidence your compliance, and lead your team with confidence.

FREE RESOURCE | 2026 EDITION | [KAJIDORICOLLECTIVE.CO.UK](https://www.kajidori.co.uk)

Prepared by Yeukai Kajidori | Founder, The Kajidori Collective | 20+ Years in Health & Social Care Leadership

How to Use This Checklist

CQC inspections are not pass or fail events. They are a structured assessment of whether your service consistently delivers safe, effective, caring, responsive, and well-led care. This checklist mirrors the five key questions inspectors use — and the kinds of evidence they look for when they walk through your door.

Work through each section with your senior team. For every item, tick the box if you can evidence it confidently. If you cannot, that gap is your priority. Do not wait for an inspection to find out what you already know is missing.

"Outstanding services do not happen by accident. They are built through deliberate systems, consistent leadership, and a culture that holds quality as non-negotiable."

— Yeukai Kajidori, The Kajidori Collective

About The Kajidori Collective

The Kajidori Collective is a UK-based centre of excellence in health and social care leadership. We work with care providers, NHS organisations, and sector leaders to raise standards, strengthen governance, and build the leadership capability that drives Outstanding CQC outcomes.

CQC Compliance Audits	Mental Health & Complex Needs Training — from £2,500/day
AI Integration Workshops — from £1,600	Strategic Consulting — from £1,200/day
Leadership Mentoring — £950/participant	Kajidori Conversations Podcast — 580+ episodes

01

CQC KEY QUESTION 1 OF 5

SAFE

Inspectors assess whether people are protected from abuse, avoidable harm, and neglect. This section covers your safeguarding systems, medication management, and risk frameworks.

- ☐ **Safeguarding policy is current, staff-trained, and referenced in practice**
Evidence: training records, supervision notes, incident logs
- ☐ **Medication management system is audited monthly with no unresolved errors**
Evidence: MAR charts, audits, pharmacist reviews
- ☐ **Risk assessments are individual, updated after incidents, and reviewed quarterly**
Evidence: care plan risk sections, review dates, sign-offs
- ☐ **Accident and incident reporting is timely, investigated, and action-evidenced**
Evidence: incident forms, investigation outcomes, learning records
- ☐ **Infection prevention and control policy is current and embedded in daily practice**
Evidence: IPC audits, cleaning schedules, staff training records
- ☐ **Lone working and staff safety protocols are documented and followed**
Evidence: lone working policy, check-in logs, risk assessments
- ☐ **Deprivation of Liberty Safeguards (DoLS) applications are up to date where required**
Evidence: DoLS register, authorisation paperwork, mental capacity assessments
- ☐ **Recruitment processes include full DBS checks, references, and right-to-work verification**
Evidence: staff files, DBS certificates, safer recruitment checklist

02

CQC KEY QUESTION 2 OF 5

EFFECTIVE

This question focuses on whether people's needs are assessed, care is evidence-based, and outcomes are positive. Staff competency and multi-agency working are central.

- ☐ **Care plans are personalised, co-produced with the person, and reviewed regularly**
Evidence: signed care plans, review meeting notes, resident/family input records
- ☐ **Staff receive structured induction, role-specific training, and annual refreshers**
Evidence: training matrix, certificates, competency sign-offs
- ☐ **Mental capacity assessments are completed appropriately and documented correctly**
Evidence: MCA assessments, best interest meeting records, decision logs
- ☐ **Nutrition and hydration monitoring is in place for all people at risk**
Evidence: MUST scores, food and fluid charts, GP referrals where indicated
- ☐ **The service works effectively with GPs, community nurses, and specialist services**
Evidence: referral records, MDT meeting notes, communication logs
- ☐ **Outcomes data is collected, analysed, and used to drive service improvement**
Evidence: outcome tracking tools, quality reports, improvement plans
- ☐ **Staff supervision is regular, structured, and documented**
Evidence: supervision schedule, completed supervision records, appraisal records

03

CQC KEY QUESTION 3 OF 5

CARING

Inspectors look at how staff treat people — whether dignity is upheld, independence is promoted, and people feel involved in decisions about their care.



People are treated with dignity and respect in all interactions and environments

Evidence: observation, feedback surveys, complaints analysis



People are supported to make their own decisions and maintain independence

Evidence: care plans, activity records, family feedback



Emotional wellbeing and mental health needs are recognised and responded to

Evidence: wellbeing assessments, care notes, specialist referrals



Privacy is maintained consistently — in personal care, conversations, and records

Evidence: policy, staff training records, environment checks



People and families are kept informed and involved in care planning

Evidence: family meeting records, signed care plans, communication logs



End-of-life care is planned sensitively and aligned with the person's wishes

Evidence: advance care plans, DNACPR records, EOLC training evidence



Staff demonstrate compassion and are supported to maintain their own wellbeing

Evidence: staff wellbeing surveys, supervision records, wellbeing initiatives

04

CQC KEY QUESTION 4 OF 5

RESPONSIVE

This section assesses whether the service is organised to meet people's individual needs and responds promptly and appropriately when things go wrong.

- ☐ **Complaints are logged, investigated within timescales, and resolved with evidence**
Evidence: complaints register, investigation records, response letters
- ☐ **Care is flexible and adapts to changing needs without delay**
Evidence: care review records, unplanned changes documented, crisis plans
- ☐ **People's cultural, religious, and communication needs are actively accommodated**
Evidence: personal profiles, communication plans, dietary/activity records
- ☐ **Waiting times for reviews, assessments, and responses are within acceptable limits**
Evidence: referral logs, response time tracking, review scheduling records
- ☐ **Activities and social engagement are meaningful, varied, and person-led**
Evidence: activity planner, participation records, resident feedback
- ☐ **The service responds to learning from incidents, complaints, and audits**
Evidence: learning logs, policy updates, staff briefing records
- ☐ **Discharge and transition planning is proactive and involves all relevant parties**
Evidence: discharge plans, handover records, family communication logs

05

CQC KEY QUESTION 5 OF 5

WELL-LED

The leadership question is the one that underpins everything else. Inspectors assess whether there is a clear vision, strong governance, and a culture of continuous improvement.

- ☐ **The Registered Manager is visible, accessible, and actively leads quality improvement**
Evidence: manager's audit schedule, walk-around records, team meeting minutes
- ☐ **A clear governance framework is in place and audits are completed on schedule**
Evidence: audit calendar, completed audits, action plans with sign-off dates
- ☐ **The service has a written vision and values that are understood and lived by staff**
Evidence: staff induction records, team meeting references, observation
- ☐ **Staff feel safe to raise concerns and there is a culture free from blame**
Evidence: speaking-up policy, whistleblowing records, staff survey results
- ☐ **Quality assurance processes identify issues before they escalate**
Evidence: QA reports, trend analysis, proactive improvement plans
- ☐ **Notifications to CQC are submitted on time for all notifiable events**
Evidence: notifications log, confirmation receipts, policy and procedure
- ☐ **The provider statement of purpose is current and accurate**
Evidence: signed statement of purpose, CQC registration record
- ☐ **There is a business continuity plan that is regularly tested**
Evidence: BCP document, test records, staff awareness training

Ready to Close Your Gaps?

This checklist tells you where you are. A Kajidori Collective audit tells you exactly what to do next — with a tailored action plan, evidenced recommendations, and a clear path to an Outstanding CQC outcome.

BOOK A FREE 30-MINUTE STRATEGY CALL

<https://tfft.io/kMpCYo2>

kajidoricollective@gmail.com

kajidoricollective.co.uk

Kajidori Conversations Podcast

© 2026 The Kajidori Collective. All rights reserved. This resource is intended for professional use within health and social care settings. It does not constitute legal or regulatory advice.